PharmEasy Labs

Report for Yogendar Kadwasra(30Y/M)

Tests asked Comprehensive Full Body Checkup With Vitamin D And B12 - 3, Test + 1 Others

Test date 08 Feb 2025

Report status Complete Report

REPORT VERIFIED VERIFIED BY DOCTOR

6 STEP quality control to ensure 100% report accuracy

- Qualified and trained technicians
- Temperature-controlled containers to store samples
- Strict quality checks on samples before processing
- Regular monitoring of lab analyzers by experts
- Assured machine inspection on a daily basis
- Verified reports by qualified pathologists

25+ Years of Trust & Experience | NABL Accredited Labs | 100+ Crore Samples Processed

PharmEasy Labs

Name : YOGENDAR KADWASRA(30Y/M)

Ref. By : SELF

ADDRESS :

HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR

Report Availability Summary

Full Report Available

Note : This is summary page. Please refer to the table below for the details

Test	Report Status
CARDIAC RISK MARKERS	Available
Comprehensive Full Body Checkup With Vitamin D and B12 - 3	Available
25-OH VITAMIN D (TOTAL)	Available
COMPLETE URINE ANALYSIS	Available
FASTING BLOOD SUGAR(GLUCOSE)	Available
HbA1c	Available
HEMOGRAM - 6 PART (DIFF)	Available
IRON	Available
KIDPRO	Available
LIPID PROFILE	Available
LIVER FUNCTION TESTS	Available
TOTAL THYROXINE (T4)	Available
TOTAL TRIIODOTHYRONINE (T3)	Available
TSH - ULTRASENSITIVE	Available
VITAMIN B-12	Available
TESTOSTERONE	Available

Note : Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by : Thyrocare Technologies Ltd.

PharmEasy Labs

NAME : YOGENDAR KADWASRA(30Y/M)
REF. BY : SELF
TEST ASKED : CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE

HOME COLLECTION :
HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c - (HPLC)			
	H.P.L.C	5.6	%

Bio. Ref. Interval. :

Bio. Ref. Interval.: As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	114	mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Sample Collected on (SCT) : 08 Feb 2025 10:58
Sample Received on (SRT) : 08 Feb 2025 14:07
Report Released on (RRT) : 08 Feb 2025 15:42
Sample Type : EDTA Whole Blood
Labcode : 0802080474/DG007
Barcode : DJ437084

Saakshi

Dr Saakshi Mittal MD(Path)

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Clinically Tested by :Thyrocare Technologies Ltd

PharmEasy Labs

NAME : YOGENDAR KADWASRA(30Y/M)
REF. BY : SELF
TEST ASKED : CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE

HOME COLLECTION :
HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	**SLS-Hemoglobin Method**	**12.7**	**g/dL**	**13.0-17.0**
Hematocrit (PCV)	**CPH Detection**	**39.1**	**%**	**40.0-50.0**
Total RBC	HF & EI	5.32	X 10^6/µL	4.5-5.5
Mean Corpuscular Volume (MCV)	**Calculated**	**73.5**	**fL**	**83.0-101.0**
Mean Corpuscular Hemoglobin (MCH)	**Calculated**	**23.9**	**pq**	**27.0-32.0**
Mean Corp.Hemo. Conc (MCHC)	Calculated	32.5	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	**Calculated**	**46.5**	**fL**	**39-46**
Red Cell Distribution Width (RDW - CV)	**Calculated**	**18.2**	**%**	**11.6-14**
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	251.4	-	*Refer Note below
MENTZER INDEX	Calculated	13.8	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	7.39	X 10^3 / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	40.4	%	40-80
Lymphocytes Percentage	**Flow Cytometry**	**52.8**	**%**	**20-40**
Monocytes Percentage	Flow Cytometry	3.4	%	2-10
Eosinophils Percentage	Flow Cytometry	2	%	1-6
Basophils Percentage	Flow Cytometry	1.1	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	2.99	X 10^3 / µL	2.0-7.0
Lymphocytes - Absolute Count	**Calculated**	**3.9**	**X 10^3 / µL**	**1.0-3.0**
Monocytes - Absolute Count	Calculated	0.25	X 10^3 / µL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.08	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.15	X 10^3 / µL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10^3 / µL	0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	351	X 10^3 / µL	150-410
Mean Platelet Volume (MPV)	Calculated	11.2	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	14.1	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	35.2	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.39	%	0.19-0.39

Remarks : Alert!!! RBCs:Moderate anisocytosis mild poikilocytosis. Predominantly microcytic hypochromic cells with ovalocytes & elliptocytes. Platelets:Appear adequate in smear.

***Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)
(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height**)**

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Labcode : 0802080474/DG007
Barcode : DJ437084

Saakshi

Dr Saakshi Mittal MD(Path)

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Clinically Tested by :Thyrocare Technologies Ltd

PharmEasy Labs

NAME	: YOGENDAR KADWASRA(30Y/M)	**HOME COLLECTION :**	
REF. BY	: SELF	HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR	
TEST ASKED	: CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE		

TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	91.35	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.

Method:- GOD-PAP METHOD

Sample Collected on (SCT)	: 08 Feb 2025 10:58
Sample Received on (SRT)	: 08 Feb 2025 14:07
Report Released on (RRT)	: 08 Feb 2025 15:09
Sample Type	: FLUORIDE PLASMA
Labcode	: 0802080559/DG007
Barcode	: DI661109

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NAME : YOGENDAR KADWASRA(30Y/M)
REF. BY : SELF
TEST ASKED : CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE

HOME COLLECTION :
HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
Complete Urinogram				
Physical Examination				
VOLUME	Visual Determination	4	mL	-
COLOUR	Visual Determination	PALE YELLOW	-	Pale Yellow
APPEARANCE	Visual Determination	CLEAR	-	Clear
SPECIFIC GRAVITY	pKa change	1.02	-	1.003-1.030
PH	pH indicator	5.5	-	5-8
Chemical Examination				
URINARY PROTEIN	PEI	ABSENT	mg/dL	Absent
URINARY GLUCOSE	GOD-POD	ABSENT	mg/dL	Absent
URINE KETONE	Nitroprusside	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	Diazo coupling	ABSENT	mg/dL	Absent
UROBILINOGEN	Diazo coupling	Normal	mg/dL	<=0.2
BILE SALT	Hays sulphur	ABSENT	-	Absent
BILE PIGMENT	Ehrlich reaction	ABSENT	-	Absent
URINE BLOOD	Peroxidase reaction	ABSENT	-	Absent
NITRITE	Diazo coupling	ABSENT	-	Absent
LEUCOCYTE ESTERASE	Esterase reaction	ABSENT	-	Absent
Microscopic Examination				
MUCUS	Microscopy	ABSENT	-	Absent
RED BLOOD CELLS	Microscopy	ABSENT	cells/HPF	0-5
URINARY LEUCOCYTES (PUS CELLS)	Microscopy	ABSENT	cells/HPF	0-5
EPITHELIAL CELLS	Microscopy	ABSENT	cells/HPF	0-5
CASTS	Microscopy	ABSENT	-	Absent
CRYSTALS	Microscopy	ABSENT	-	Absent
BACTERIA	Microscopy	ABSENT	-	Absent
YEAST	Microscopy	ABSENT	-	Absent
PARASITE	Microscopy	ABSENT	-	Absent

(Reference : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase**)**

Sample Collected on (SCT) : 08 Feb 2025 10:58
Sample Received on (SRT) : 08 Feb 2025 16:24
Report Released on (RRT) : 08 Feb 2025 18:38
Sample Type : URINE
Labcode : 0802092306/DG007
Barcode : CZ693227

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NAME	: YOGENDAR KADWASRA(30Y/M)	**HOME COLLECTION :**	
REF. BY	: SELF	HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR	
TEST ASKED	: CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE		

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	E.C.L.I.A	32.5	ng/mL

Bio. Ref. Interval. :-

Deficiency : <=20 ng/ml || Insufficiency : 21-29 ng/ml
Sufficiency : >= 30 ng/ml || Toxicity : >100 ng/ml

Clinical Significance:
Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.
Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.
Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%
Kit Validation Reference : Holick M. Vtamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002:9(1)87-98.

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

Sample Collected on (SCT)	: 08 Feb 2025 10:58
Sample Received on (SRT)	: 08 Feb 2025 14:10
Report Released on (RRT)	: 08 Feb 2025 17:32
Sample Type	: SERUM
Labcode	: 0802080924/DG007
Barcode	: DC997192

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Clinically Tested by :Thyrocare Technologies Ltd - (NABL accredited)

PharmEasy Labs

NAME : YOGENDAR KADWASRA(30Y/M)
REF. BY : SELF
TEST ASKED : CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE

HOME COLLECTION :
HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR

TEST NAME	TECHNOLOGY	VALUE	UNITS
APOLIPOPROTEIN - A1 (APO-A1)	IMMUNOTURBIDIMETRY	116	mg/dL

Bio. Ref. Interval. :

Male : 86 - 152
Female : 94 - 162
Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

APOLIPOPROTEIN - B (APO-B)	IMMUNOTURBIDIMETRY	87	mg/dL

Bio. Ref. Interval. :

Male : 56 - 145
Female : 53 - 138
Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

APO B / APO A1 RATIO (APO B/A1)	CALCULATED	0.7	Ratio

Bio. Ref. Interval. :

Male : 0.40 - 1.26
Female : 0.38 - 1.14

Clinical Significance :

- Apolipoprotein B is a more potent and independent predictor of Coronary artery disease (CAD) than LDL Cholesterol.
- Apolipoprotein A1 is one of the apoproteins of HDL and is inversely related to risk of CAD.
- The Apolipoprotein studies help in monitoring risk of restenosis in patients with myocardial infarction, Coronary bypass surgery etc.
- An increased ratio of Apo B to A1 beyond the defined normal range is indicative of CAD risk.
- All results have to be interpreted in Conjunction with clinical history and other findings.

Method : Derived from serum Apo A1 and Apo B values

Please correlate with clinical conditions.

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REF. BY	: SELF	HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR	
TEST ASKED	: CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE		

TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	IMMUNOTURBIDIMETRY	2.59	mg/L

Bio. Ref. Interval. :-

< 1.00 - Low Risk
1.00 - 3.00 - Average Risk
>3.00 - 10.00 - High Risk
> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection , active arthritis or concurrent illness.

Clinical significance:
High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

Kit Validation Reference:
1.Clinical management of laboratory date in medical practice 2003-3004, 207(2003).
2.Tietz : Textbook of Clinical Chemistry and Molecular diagnostics :Second edition :Chapter 47:Page no.1507- 1508.

Please correlate with clinical conditions.

Method:- FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

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REF. BY	: SELF	HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR	
TEST ASKED	: CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE		

TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12	E.C.L.I.A	313	pg/mL

Bio. Ref. Interval. :-

Normal: 197-771 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L.Clinical laborator Diagnostics : Use and Assessment of Clinical laboratory Results 1st Edition,TH Books-Verl-Ges,1998:424-431

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Compititive Immunoassay

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TEST ASKED	: CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE		

TEST NAME	TECHNOLOGY	VALUE	UNITS
Lipoprotein (a) [Lp(a)]	IMMUNOTURBIDIMETRY	21.42	mg/dL

Bio. Ref. Interval. :-

Adults : < 30.0 mg/dl

Clinical Significance:
Determination of LPA may be useful to guide management of individuals with a family history of CHD or with existing disease. The levels of LPA in the blood depends on genetic factors; The range of variation in a population is relatively large and hence for diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:
Precision %CV :- Intra assay %CV- 4.55% , Inter assay %CV-0.86 %

Kit Validation Reference:
Tietz NW,Clinical Guide to Laboratory Tests Philadelphia WB. Saunders 1995 : 442-444

Please correlate with clinical conditions.

Method:- LATEX ENHANCED IMMUNOTURBIDIMETRY

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Sample Type	: SERUM
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REF. BY	: SELF	HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR	
TEST ASKED	: CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE		

TEST NAME	TECHNOLOGY	VALUE	UNITS
TESTOSTERONE	E.C.L.I.A	407	ng/dL

Bio. Ref. Interval. :-

280 - 800

Clinical Significance: Clinical evaluation of serum testosterone, along with serum LH, assists in evaluation of Hypogonadal males. Major causes of lowered testosterone in males include Hypogonadotropic hypogonadism, testicular failure Hyperprolactinema, Hypopituitarism some types of liver and kidney diseases and critical illness.

Specifications: Precision: Intra assay (%CV): 11.50 %, Inter assay (%CV): 5.70%; Sensitivity: 7 ng/dL.
Kit Validation Reference: Wilson JD Foster DW (Eds) Williams Textbook of Endocrinology 8th Edition WB Saunders Piladelphia Pennsylvania.

Note : The Biological Reference Range mentioned is specific to the age group and gender. Kindly correlate clinically.

Please correlate with clinical conditions.

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HOME COLLECTION :
HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	PHOTOMETRY	164	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**29**	**mg/dL**	**40-60**
LDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**107.79**	**mg/dL**	**< 100**
TRIGLYCERIDES	PHOTOMETRY	42	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	**CALCULATED**	**5.7**	**Ratio**	**3 - 5**
TRIG / HDL RATIO	CALCULATED	1.46	Ratio	< 3.12
LDL / HDL RATIO	**CALCULATED**	**3.7**	**Ratio**	**1.5-3.5**
HDL / LDL RATIO	**CALCULATED**	**0.27**	**Ratio**	**> 0.40**
NON-HDL CHOLESTEROL	CALCULATED	135.2	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	8.38	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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Clinically Tested by :Thyrocare Technologies Ltd

PharmEasy Labs
Registered Address: DOCON TECHNOLOGIES PRIVATE LIMITED, Office No 208, 209 & 210 Second floor, A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka, Andheri (East), Mumbai - 400072
Lab Address: Plot No.428, 2nd flr, Phase-IV, Udyog| Vihar, Gurgaon, Haryana - 122015
7022 000 900

PharmEasy Labs

NAME : YOGENDAR KADWASRA(30Y/M)
REF. BY : SELF
TEST ASKED : CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE

HOME COLLECTION :
HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	PHOTOMETRY	86.2	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.4	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.1	mg/dL	< 0.3
BILIRUBIN (INDIRECT)	CALCULATED	0.3	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	17	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	33.1	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	38.5	U/L	< 45
SGOT / SGPT RATIO	CALCULATED	0.86	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.85	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.4	gm/dL	3.2-4.8
SERUM GLOBULIN	**CALCULATED**	**3.45**	**gm/dL**	**2.5-3.4**
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.28	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Vanadate Oxidation
BILD - Vanadate Oxidation
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

Sample Collected on (SCT) : 08 Feb 2025 10:58
Sample Received on (SRT) : 08 Feb 2025 14:10
Report Released on (RRT) : 08 Feb 2025 17:32
Sample Type : SERUM
Labcode : 0802080924/DG007
Barcode : DC997192

Saakshi

Dr Saakshi Mittal MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd

PharmEasy Labs
Registered Address: DOCON TECHNOLOGIES PRIVATE LIMITED, Office No 208, 209 & 210 Second floor, A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka, Andheri (East), Mumbai - 400072
Lab Address: Plot No.428, 2nd flr, Phase-IV, Udyog| Vihar, Gurgaon, Haryana - 122015
7022 000 900

PharmEasy Labs

NAME	:	YOGENDAR KADWASRA(30Y/M)
REF. BY	:	SELF
TEST ASKED	:	CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE

HOME COLLECTION :
HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR

TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON	**PHOTOMETRY**	**60**	**µg/dL**

Bio. Ref. Interval. :

Male : 65 - 175
Female : 50 - 170

Method : Ferrozine method without deproteinization

Please correlate with clinical conditions.

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PharmEasy Labs

NAME : YOGENDAR KADWASRA(30Y/M)
REF. BY : SELF
TEST ASKED : CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE

HOME COLLECTION :
HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	16	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.81	mg/dL	0.72-1.18
BUN / Sr.CREATININE RATIO	CALCULATED	19.75	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	34.24	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	42.27	Ratio	< 52
CALCIUM	PHOTOMETRY	9.71	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	5.3	mg/dL	4.2 - 7.3

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

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NAME	:	YOGENDAR KADWASRA(30Y/M)
REF. BY	:	SELF
TEST ASKED	:	CARDIAC RISK MARKERS,Comprehensive Full Body Checkup With Vitamin D and B12 - 3,TESTOSTERONE

HOME COLLECTION :
HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	103	ng/dL	80-200
TOTAL THYROXINE (T4)	E.C.L.I.A	5.45	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	E.C.L.I.A	1.37	µIU/mL	0.54-5.30

Comments : IF NOT ON DRUGS SUGGESTED FT3 & FT4 ESTIMATION

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3 - Fully Automated Electrochemiluminescence Compititive Immunoassay
T4 - Fully Automated Electrochemiluminescence Compititive Immunoassay
USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Disclaimer :

Results should always be interpreted using the reference range provided by the laboratory that performed the test.
Different laboratories do tests using different technologies, methods and using different reagents which may cause difference
In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports.
To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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HOUSE NO J 678 MEN BHATTA ROAD SARUK NAGAR GULLY NO 14 NORTH DELHI BADLI DELHI NCR

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	119	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- CKD-EPI Creatinine Equation

~~ End of report ~~

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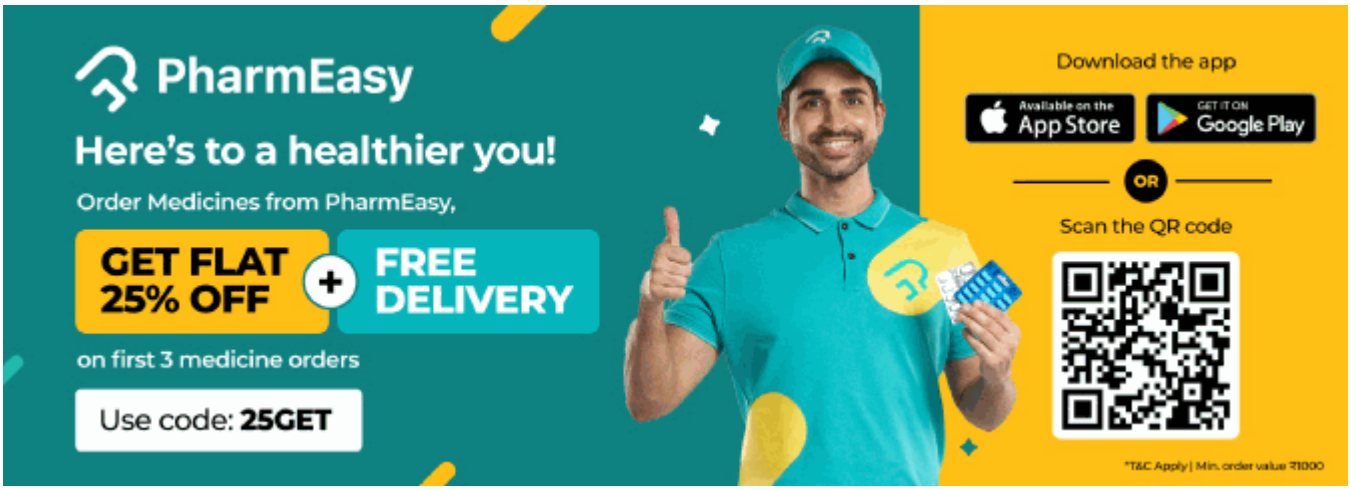
CONDITIONS OF REPORTING

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Docon Technologies Private Limited,Thyrocare Technologies Limited and its employees/representatives do not assume any liability,responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

EXPLANATIONS

- **Name -** The name is as declared by the client and recorded by the personnel who collected the specimen.
- **Ref.By -** The name of the doctor who has recommended testing as declared by the client.
- **Labcode -** This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode -** This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCT -** Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT -** Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT -** Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range -** Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- For suggestions, complaints or feedback, write to us at grievance-office@docon.co.in or call us on 7022000900.